

NEJM AI + Health Affairs Submission Guide

Manasa Jampani — Paper 4 (Inside the Payer AI Black Box)

Primary Target: NEJM AI

URL: <https://ai.nejm.org/author-center>

Scope: Clinical, operational, and policy implications of AI in medicine and healthcare

Why Paper 4 Fits: NEJM AI specifically seeks healthcare AI insider analysis with clinical implications — Manasa's paper is exactly this category. No academic researcher can provide this vantage point.

NEJM AI Submission Steps

1. Prepare Your Manuscript

Format requirements: - Word count: 2,500–4,000 words for Original Investigation; 1,500–2,500 for Perspective - Recommend: **Perspective** format — NEJM AI's "Perspective" is specifically for expert viewpoints and insider analysis, ideal for Paper 4 - References: Vancouver/NLM format - Abstract: Structured (Background, Methods/Approach, Findings, Interpretation) for Original Investigation; unstructured for Perspective

2. Create Author Account

Navigate to: <https://ai.nejm.org/author-center> → "Submit a Manuscript"
System: Editorial Manager (same as NEJM main journal)
Register with: founders@ardiahealthlabs.com

3. Submission Form

Article type: **Perspective** (for expert insider analysis)
OR: **Original Investigation** (if framed as a structured analysis with methodology section)

Title:

Inside the Payer AI Black Box: Operational Analysis of Clinical AI Denial Systems in Independent Laboratory Billing and Countermeasure Architecture

Cover Letter Template:

Dear NEJM AI Editors,

I am submitting "Inside the Payer AI Black Box: Operational Analysis of Clinical AI Denial Systems in Independent Laboratory Billing and

Countermeasure Architecture" for consideration as a Perspective in NEJM AI.

This paper provides what I believe is the only published insider analysis of how payer AI denial systems operate in the molecular diagnostic billing domain. As the Co-Founder of Ardia Health Labs and a professional with 5+ years of healthcare IT experience at United HealthGroup and Interwell Health – organizations on the payer side of the clinical laboratory revenue equation – I have direct operational visibility into the payer AI denial system configuration that produces the documented 35.3% molecular diagnostic denial rate (XiFin 2024, 20M+ claims) and 2.76× denial disparity for independent laboratories (JAMA Network Open 2025, n=29,919).

No other author can provide this vantage point. The clinical operations knowledge that governs payer AI denial logic is proprietary, not publicly documented, and visible only from within payer clinical operations. This paper makes it visible to the academic and policy community for the first time.

The paper characterizes four categories of payer AI denial logic, analyzes the information asymmetry that produces the documented denial disparity, and derives countermeasure architecture implications. The findings have direct policy implications for the Texas SB 1188 and TRAIGA AI governance frameworks, which I argue should apply symmetrically to payer denial AI systems.

The manuscript has not been submitted elsewhere. I declare no conflicts of interest beyond my role as co-founder of a company developing provider-side countermeasure technology – a role that provides my unique qualifications for writing this paper.

Sincerely,
Manasa Jampani
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4. medRxiv Preprint First

Before journal submission, post to medRxiv for immediate publication record:

URL: <https://www.medrxiv.org/submit>

Category: Health Informatics

Timeline: Live in 24–48 hours after moderation

EB-2 NIW value: medRxiv preprints establish expert authority in the field and begin citation accumulation

5. After NEJM AI — Secondary Targets

If NEJM AI declines (typical first-submission acceptance rate ~10%):

Journal	Why
Health Affairs	Policy-oriented; payer AI transparency angle fits perfectly
American Journal of Managed Care	Managed care / payer operations readership
Health Services Research	Operational analysis of healthcare systems
JAMIA	Healthcare informatics; technical analysis fits

medRxiv Submission for Immediate Publication Record

URL: <https://www.medrxiv.org>

Timeline: 24–48 hours (faster than arXiv for health sciences)

Category: Health Informatics → Healthcare Management

Why medRxiv first: - Same day as submission: claim a publication date for immigration evidence - medRxiv preprints are indexed by Google Scholar within days - NEJM AI and all target journals allow medRxiv preprints (check each journal's preprint policy before submission)

Steps: 1. Create account at medrxiv.org 2. Upload PDF of Paper 4 3. Select category: "Health Informatics" 4. Fill title, abstract, author information 5. Submit for moderation (24–48 hours to live status) 6. Save your medRxiv DOI — this is your immigration evidence document

Health Affairs Submission (Secondary)

URL: <https://www.healthaffairs.org/about/for-authors>

Scope: Health policy analysis; payer transparency and AI governance angle is ideal

Why Paper 4 fits: Health Affairs specifically publishes insider analysis of healthcare system operations; Manasa's payer-insider perspective is exactly the type of analysis they publish.

Cover letter angle for Health Affairs: > Frame Paper 4 as a policy argument: payer AI denial systems lack the transparency and accountability required by Texas SB 1188 and TRAIGA. The paper provides evidence — from insider operational knowledge — of the information asymmetry that current payer AI opacity creates. Health Affairs audience is health policy

makers, regulators, and executives — exactly the right audience for payer AI transparency arguments.

Word limits: Health Affairs: 2,000–3,500 words for Health Affairs Blog; 3,500–6,000 for full research article
Paper 4 as drafted: ~4,500 words → suitable for full article or trim to blog post for speed

LinkedIn Articles for Immigration Evidence

Publish Before Journal Submission: LinkedIn articles indexed by Google count as “published material” for NIW evidence and can be posted immediately — no review process.

Recommended Articles for Manasa:

Article 1: Title: “Why Payer AI Denial Systems Are the Invisible Hand Killing Independent Labs” Content: High-level version of Paper 4’s thesis — the information asymmetry argument, quantified with JAMA/XiFin statistics. Accessible to healthcare business audience. Target: 800–1,200 words; post in healthcare AI, RCM, and revenue cycle LinkedIn groups

Article 2: Title: “I Spent 5 Years on the Payer Side. Here’s What Independent Labs Don’t Know About Claim Denials.” Content: Personal narrative angle — insider knowledge, what she saw, why it matters. First-person for authenticity and engagement. Target: 600–1,000 words

Article 3: Title: “PAMA 2027 + 35% Denial Rates: Why Independent Labs Need AI Countermeasures in the Next 12 Months” Content: Urgency argument; PAMA timeline; what happens without AI-driven recovery Target: 700–1,000 words

Publication value for NIW: Google indexes LinkedIn articles. Print each article’s Google search result showing your name, title, URL, and date — this is “published material about the person” or “publications by the person” evidence for the NIW petition.

Building a Citation Network

After Paper 4 is posted (medRxiv or NEJM AI):

1. **Notify the JAMA Network Open Georgetown team** — your paper cites and extends their 2025 study. Researchers appreciate notification; they may cite you in turn.

2. **Notify XiFin** — your paper cites their 2024 Payor Denial Impact Report. XiFin has an industry relations team; they may amplify your paper.
3. **Post to LinkedIn and tag:** @AMIA, @HIMSS, @AmerMedInformatics, relevant healthcare AI groups.
4. **ResearchGate profile** — create and link your medRxiv/NEJM AI paper; increases discoverability and citation accumulation.

Citations accumulate in Google Scholar — a growing citation count strengthens the NIW Prong 2 argument over time (demonstrating that the field recognizes your expertise).